

Medication Reconciliation

Policy Number	POL-PH-034	Owner / Department	Pharmacy / Nursing
Effective Date	2024-01-08	Version	2.2
Last Revised	2026-03-05	Next Review	2028-03-05
Approved By	Pharmacy & Therapeutics Committee	Applies To	All prescribers, pharmacists, and nurses responsible for medication histories and orders at admission, transfer, and discharge

1. Purpose

To prevent medication discrepancies — omissions, duplications, dosing errors, and interactions — by comparing the medications a patient is taking against new orders at every transition of care, in compliance with The Joint Commission National Patient Safety Goal NPSG.03.06.01.

2. Definitions

- **Medication reconciliation** — the process of creating the most accurate list of all medications a patient is taking and comparing it against new orders.
- **Best possible medication history (BPMH)** — a history obtained using at least one reliable source beyond the patient interview.
- **Transition of care** — admission, transfer between levels of care or services, and discharge.
- **Discrepancy** — any unexplained difference between the medication history and current orders.

3. Policy and Procedure

A medication history is obtained and reconciled at admission, at every transfer, and at discharge. The patient or family receives a written, reconciled medication list at discharge with instructions to share it with their next providers.

1. Collect a BPMH using the patient/family plus at least one additional source (pharmacy fill history, prior records, or provider).
2. Document name, dose, route, frequency, and last dose for each medication, including OTCs, supplements, and inhalers.
3. Compare the history against admission/transfer orders and resolve every discrepancy with the prescriber.
4. At discharge, reconcile home, hospital, and new medications into a single list and explain changes to the patient.
5. Provide the reconciled list to the patient and the next provider of care.

IMPORTANT — Every unexplained discrepancy identified during reconciliation must be resolved with the prescriber before the next medication pass; a high-alert medication discrepancy must be escalated to the prescriber immediately rather than carried forward.

4. Reconciliation Points and Required Actions

Transition	Required action
Admission	Obtain BPMH; reconcile against admission orders within 24 hours.
Transfer	Re-reconcile when level of care or service changes.
Discharge	Produce one reconciled list; counsel patient; send to next provider.
High-alert meds	Resolve any discrepancy with the prescriber immediately.

5. Severity & Reporting

Classify reconciliation failures on the POL-RM-013 scale and grade resulting errors by the NCC MERP index per POL-PS-003.

Severity	Example	Notify / timeframe
SEV-1	Omitted anticoagulant at discharge contributing to fatal thrombosis.	Provider, Nursing Supervisor & Risk Mgmt within 1 hour.
SEV-2	Duplicate therapy or omitted critical drug causing harm needing intervention.	Provider & Risk Mgmt within 4 hours.
SEV-3 / Near Miss	Discrepancy reaching patient without harm; omission caught at pharmacist review.	Event report within 24 hours.

6. Documentation Requirements

- Sources used to build the BPMH and who obtained it.
- Full medication list with dose, route, frequency, and last dose.
- Each discrepancy identified, its resolution, and prescriber contacted.
- Discharge reconciled list given to the patient and sent to the next provider.

7. References

- The Joint Commission NPSG.03.06.01 — Maintain and communicate accurate patient medication information.
- CMS Conditions of Participation, 42 CFR 482.25 (pharmaceutical services).
- ISMP guidance on medication reconciliation.
- Related: POL-RM-013 (severity & reporting); POL-PS-003 (medication-error reporting).